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Impetigo

Patient Group Direction, version 011, issued 30 September 2026. Three arms: fusidic acid 2% cream, flucloxacillin oral suspension, and a macrolide where penicillin-allergic.

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Choosing the arm

- **LOCALISED non-bullous impetigo, a few lesions in one area: fusidic acid 2% cream.**
- **WIDESPREAD non-bullous impetigo, BULLOUS impetigo, or failure of topical treatment: an oral antibiotic.**
- **Where an oral antibiotic is needed and the patient is NOT penicillin-allergic: flucloxacillin.**
- **Where an oral antibiotic is needed and the patient IS penicillin-allergic: clarithromycin, or erythromycin in pregnancy.**
- **DO NOT COMBINE a topical and an oral antibiotic. NICE is explicit on this and it is a common error.**
- **Courses are 5 DAYS.**

Hygiene advice, for every patient

- Impetigo is contagious. Wash hands after touching the lesions and after applying any cream.
- Do not share towels, flannels or bedding until the lesions have crusted over or healed.
- Keep the lesions covered where practical, and discourage picking or scratching.
- **Stay away from school or nursery until the lesions are crusted and dry, or for 48 hours after starting an antibiotic.**
- Keep fingernails short.

Fusidic acid 2% cream, for localised non-bullous impetigo

Summary of NICE guidance for impetigo

Impetigo: antimicrobial prescribing, NICE guideline NG153, published 26 February 2020; minor wording changes January 2022 to reflect updated advice on the use of macrolides in pregnancy. Summarised 9 September 2026.

What impetigo is and how it presents

NICE describes impetigo as a contagious bacterial infection of the superficial layers of the skin.

The most common bacterial pathogen is Staphylococcus aureus, although infection with Streptococcus pyogenes, or a combination of both pathogens, is also seen.

Impetigo affects all age groups but is most common in young children.

Non-bullous impetigo is characterised by thin-walled vesicles or pustules that rupture quickly, forming a golden-brown crust.

Bullous impetigo is characterised by fluid-filled vesicles and blisters, often over 1 cm in diameter, that rupture leaving a thin, flat, yellow-brown crust.

NICE states that clinical judgement should be used to decide whether impetigo is localised or widespread.

Localised non-bullous impetigo: hydrogen peroxide first

For people with localised non-bullous impetigo who are not systemically unwell or at high risk of complications, consider hydrogen peroxide 1% cream, applied two or three times a day for 5 days.

NICE notes that other topical antiseptics are available for superficial skin infections but no evidence was found for using them to treat impetigo.

If hydrogen peroxide 1% cream is unsuitable (for example, if the impetigo is around the eyes) or ineffective, offer a short course of a topical antibiotic.

The first-choice topical antibiotic is fusidic acid 2%, applied three times a day for 5 days, with mupirocin 2% three times a day for 5 days as the alternative if fusidic acid resistance is suspected or confirmed.

NICE warns that extended or recurrent use of topical fusidic acid or mupirocin may increase the risk of antimicrobial resistance.

Widespread, bullous or higher-risk impetigo: when an oral antibiotic is indicated

For widespread non-bullous impetigo in people who are not systemically unwell or at high risk of complications, offer a short course of either a topical or an oral antibiotic; NICE states both are effective.

In choosing between topical and oral treatment, take into account the person's preferences, the practicalities of application (particularly to large areas), possible adverse effects, and previous use of topical antibiotics, because resistance can develop rapidly with extended or repeated use.

Offer a short course of an oral antibiotic to all people with bullous impetigo, and to people with non-bullous impetigo who are systemically unwell or at high risk of complications.

Do not offer combination treatment with a topical and an oral antibiotic.

If impetigo is worsening or has not improved after hydrogen peroxide, offer a topical antibiotic if it remains localised, or a topical or oral antibiotic if it has become widespread.

If impetigo is worsening or has not improved after completing a course of topical antibiotic, offer a short course of an oral antibiotic and consider sending a skin swab for microbiological testing.

Oral antibiotic choice, doses and course length

The first-choice oral antibiotic is flucloxacillin: adults 500 mg four times a day for 5 days.

Flucloxacillin doses for children (oral solution or capsules) are 62.5 mg to 125 mg four times a day at 1 month to 1 year, 125 mg to 250 mg four times a day at 2 to 9 years, and 250 mg to 500 mg four times a day at 10 to 17 years, each for 5 days.

For penicillin allergy, or if flucloxacillin is unsuitable, in people who are not pregnant, the alternative is clarithromycin: adults 250 mg twice a day for 5 days, which can be increased to 500 mg twice a day if needed for severe infections.

Clarithromycin for children aged 1 month to 11 years is weight-based: under 8 kg 7.5 mg/kg twice a day, 8 to 11 kg 62.5 mg twice a day, 12 to 19 kg 125 mg twice a day, 20 to 29 kg 187.5 mg twice a day, 30 to 40 kg 250 mg twice a day, each for 5 days; at 12 to 17 years, 250 mg twice a day for 5 days, increased to 500 mg twice a day if needed for severe infections.

For penicillin allergy in pregnancy, the alternative is erythromycin 250 mg to 500 mg four times a day for 5 days (in children, 8 to 17 years, 250 mg to 500 mg four times a day for 5 days); NICE states erythromycin is preferred if a macrolide is needed in pregnancy.

A 5-day course is usually appropriate but can be increased to 7 days based on clinical judgement, depending on the severity and number of lesions.

If meticillin-resistant Staphylococcus aureus is suspected or confirmed, consult a local microbiologist.

If flucloxacillin oral solution is not tolerated because of poor palatability, consider capsules.

Take account of local antimicrobial resistance data when available, and see the BNF or BNF for Children for dosing in specific populations such as hepatic or renal impairment, pregnancy and breastfeeding.

Reassessment, referral and seeking advice

Reassess if symptoms worsen rapidly or significantly at any time, or have not improved after completing a course of treatment.

When reassessing, take account of other possible diagnoses such as herpes simplex, any symptoms or signs suggesting a more serious illness such as cellulitis, and previous antibiotic use which may have led to resistant bacteria.

Refer to hospital people with impetigo and any symptoms or signs suggesting a more serious illness or condition (for example, cellulitis), and people with widespread impetigo who are immunocompromised.

Consider referral or seeking specialist advice for people who have bullous impetigo (particularly babies aged 1 year and under), impetigo that recurs frequently, who are systemically unwell, or who are at high risk of complications.
For impetigo that recurs frequently, send a skin swab for microbiological testing and consider a nasal swab and starting decolonisation treatment.
If a swab has been sent, review the antibiotic choice when results are available and change it according to results if symptoms are not improving, using a narrow-spectrum antibiotic if possible.

Advice to the patient

Advise people with impetigo, and their parents or carers if appropriate, about good hygiene measures to reduce the spread of impetigo to other areas of the body and to other people.
Advise them to seek medical help if symptoms worsen rapidly or significantly at any time, or have not improved after completing a course of treatment.
NICE notes that impetigo is highly infectious and that treatment limits spread and hastens recovery, which is likely to mean less time off school, nursery or work; the guideline does not itself set out school exclusion periods.

Patient Group Direction for the supply of fusidic acid 2% cream for the treatment of localised non-bullous impetigo.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for the product supplied, the BNF or BNF for Children, and NICE NG153 on impetigo.
- ALL USERS MUST BE ABLE TO DISTINGUISH LOCALISED FROM WIDESPREAD IMPETIGO, and non-bullous from bullous. Which arm applies turns on it.
- All users must be able to take a reliable penicillin allergy history and to distinguish a true allergy from an intolerance.
- All users must be competent to weigh a child, because the paediatric macrolide dose is by weight and not by age.
- All users must know that topical and oral antibiotics are NOT to be combined for impetigo.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must understand capacity and consent, including Gillick competence and parental responsibility.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Treatment of localised non-bullous impetigo, being a small number of lesions confined to one area, in accordance with NICE NG153.
Inclusion criteria	• Non-bullous impetigo of limited extent: typically fewer than 5 lesions, or lesions confined to an area under about 5cm. • Systemic treatment not indicated, and the patient not systemically unwell. • Consent obtained, from a person with parental responsibility where the patient is under 16 and not Gillick competent. • No exclusion criterion present. • Intact or only minimally broken skin. Topical fusidic acid penetrates a superficial infection adequately; extensively broken, deeply eroded or ulcerated skin is not suitable for a topical and needs either an oral arm or assessment.
Exclusion criteria	• Known hypersensitivity to fusidic acid or to any excipient. • **Bullous impetigo. Use the oral route.** • **Widespread impetigo, more than about 5 lesions or lesions covering more than about 5cm. Use the oral route.** • **Fusidic acid resistance suspected or confirmed. NICE offers mupirocin 2% as the alternative topical; mupirocin is NOT authorised by this PGD, so refer.** **Refer, do not supply, where any of the following applies.** • **Systemically unwell: fever, malaise, lymphadenopathy, or the patient appearing unwell. Refer the same day.** • **Signs of a more serious condition, in particular cellulitis:**

	spreading redness, warmth, swelling, or pain beyond the lesions. Refer to hospital. ● **Immunocompromised, and impetigo is widespread. NICE advises hospital referral.** ● **Bullous impetigo in a baby, meaning a child aged 1 year and under (NICE CKS). Refer or seek specialist advice.** ● **Recurrent impetigo, meaning frequent repeated episodes. Refer for swabbing and consideration of decolonisation, rather than treating again.** ● Around the eye, or involving the eyelid margin, where a topical product cannot be used safely and an ophthalmic opinion may be needed. ● **Diagnostic uncertainty, or a presentation that could be herpes simplex, eczema herpeticum, or a fungal infection.** ● **A course of antibiotic already supplied for this episode under this PGD. One course per episode; treatment failure needs reassessment and a swab, not a second guess.** ● Valid informed consent not obtained, or consent from a person with parental responsibility not available where required.
Cautions	**Avoid prolonged or repeated use. Extended or recurrent use of topical fusidic acid increases the risk of antimicrobial resistance, which is why NICE positions hydrogen peroxide 1% ahead of it for localised disease.** **Hydrogen peroxide 1% cream is the NICE initial option for localised non-bullous impetigo, with a topical antibiotic reserved for where it is unsuitable, for example around the eyes, or ineffective. Hydrogen peroxide 1% is a pharmacy medicine and does not require a PGD; offer it as a P sale first where it is appropriate, and record that you did.** Do not apply to large body surface areas. Avoid contact with the eyes. If contact occurs, rinse thoroughly with water. Review if no improvement after 48 hours; treatment failure means an oral antibiotic or a swab, not a longer topical course.
Actions if the patient is excluded or declines	Explain why supply is not appropriate and what to do instead. Where the exclusion is widespread or bullous disease, move to the appropriate oral arm rather than referring, provided its criteria are met. Give the hygiene advice regardless. Document the advice and the decision.

The medicine

Name, form and strength	Fusidic acid 2% cream.
Legal category	POM.
Dose and frequency	Apply a thin layer three times a day for 5 days. Cover the lesion and about 1cm of surrounding skin.
Quantity supplied	One 15g tube. No repeat supply under this PGD.
Maximum treatment period	5 days, extended to 7 only on clinical judgement where lesions are severe or numerous, with the reason recorded.
Storage	Store at room temperature, protected from heat. Keep in the original tube with the cap closed.
Adverse effects	Common: local irritation, itching, stinging or burning on application, rash. Rare: hypersensitivity including contact dermatitis and angioedema.

Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given, and from whom. Where the patient is under 16, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment. • Patient name, address, date of birth, and the GP with whom they are registered. • **WHETHER THE IMPETIGO IS LOCALISED OR WIDESPREAD, AND BULLOUS OR NON-BULLOUS, with the number and size of lesions that led to that conclusion.** • **THE PENICILLIN ALLERGY HISTORY in the patient's own terms, and which arm was therefore used.** • **Where a macrolide was supplied to a child: THE WEIGHT IN KILOGRAMS, measured today, and the band it placed the child in.** • Pregnancy status where relevant, and how it was established. • Name, form, strength, dose, quantity and date of supply. • That hygiene advice was given, and that the patient was told topical and oral treatment are not combined. • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when treatment finished.
Disposal	Advise the patient to return any unused cream to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet provided with the product, and the hygiene advice in writing.
Counselling	• Apply a thin layer three times a day for 5 days. Wash your hands before and after. • **Do not use it for longer than the course, and do not keep the tube for next time.** • Impetigo is contagious. Wash hands after touching the lesions and after applying any cream. • Do not share towels, flannels or bedding until the lesions have crusted over or healed. • Keep the lesions covered where practical, and discourage picking or scratching. • **Stay away from school or nursery until the lesions are crusted and dry, or for 48 hours after starting an antibiotic.** • Keep fingernails short.
Follow-up	Come back if there is no improvement after 48 hours, if the lesions spread, or if you feel unwell.

Flucloxacillin oral suspension, for children aged 3 months to 17 years (under 18) who are not penicillin-allergic

Patient Group Direction for the supply of flucloxacillin 250mg/5ml oral suspension for widespread non-bullous impetigo, bullous impetigo, or failure of topical treatment, in children aged 3 months to 17 years inclusive (under 18).

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for the product supplied, the BNF or BNF for Children, and NICE NG153 on impetigo.
- ALL USERS MUST BE ABLE TO DISTINGUISH LOCALISED FROM WIDESPREAD IMPETIGO, and non-bullous from bullous. Which arm applies turns on it.
- All users must be able to take a reliable penicillin allergy history and to distinguish a true allergy from an intolerance.
- All users must be competent to weigh a child, because the paediatric macrolide dose is by weight and not by age.
- All users must know that topical and oral antibiotics are NOT to be combined for impetigo.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must understand capacity and consent, including Gillick competence and parental responsibility.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Treatment of widespread non-bullous impetigo, bullous impetigo, or impetigo that has failed topical treatment, in a child aged 3 months to 17 years inclusive (under 18) who is not penicillin-allergic.
Inclusion criteria	● Aged 3 months to 17 years inclusive (under 18). ● Widespread non-bullous impetigo, bullous impetigo, or failure of topical therapy after 48 hours. ● Able to take an oral medicine. ● **NOT penicillin-allergic. Where the child is, use the macrolide arm.** ● Consent obtained from a person with parental responsibility, or from the young person where Gillick competent. ● No exclusion criterion present.
Exclusion criteria	● **Known penicillin allergy of any type, or flucloxacillin hypersensitivity. Use the macrolide arm.** ● Cephalosporin allergy with a high risk of cross-reactivity to penicillins. ● **Previous cholestasis or jaundice associated with flucloxacillin.** ● Severe hepatic impairment, or severe renal impairment (eGFR below 30 mL/min/1.73m2). ● Under 3 months of age. Refer. ● **Adults aged 18 and over. This arm is a paediatric oral suspension and this PGD carries no adult oral flucloxacillin.**

	Refer. **Refer, do not supply, where any of the following applies.** ● **Systemically unwell: fever, malaise, lymphadenopathy, or the patient appearing unwell. Refer the same day.** ● **Signs of a more serious condition, in particular cellulitis: spreading redness, warmth, swelling, or pain beyond the lesions. Refer to hospital.** ● **Immunocompromised, and impetigo is widespread. NICE advises hospital referral.** ● **Bullous impetigo in a baby, meaning a child aged 1 year and under (NICE CKS). Refer or seek specialist advice.** ● **Recurrent impetigo, meaning frequent repeated episodes. Refer for swabbing and consideration of decolonisation, rather than treating again.** ● Around the eye, or involving the eyelid margin, where a topical product cannot be used safely and an ophthalmic opinion may be needed. ● **Diagnostic uncertainty, or a presentation that could be herpes simplex, eczema herpeticum, or a fungal infection.** ● **A course of antibiotic already supplied for this episode under this PGD. One course per episode; treatment failure needs reassessment and a swab, not a second guess.** ● Valid informed consent not obtained, or consent from a person with parental responsibility not available where required.
Cautions	**Flucloxacillin can rarely cause cholestatic hepatitis, which may present up to two months after treatment. Advise the family to seek advice for jaundice, dark urine or pale stools, including after the course has finished.** **Take on an empty stomach, an hour before food or two hours after, because food reduces absorption.** Gastrointestinal upset is common. Advise on severe or bloody diarrhoea and the possibility of Clostridioides difficile. **The suspension is unpalatable and a common reason for a course not being finished. Where the child will not take it, the macrolide arm is a reasonable alternative on grounds of unsuitability, as NICE allows.**
Actions if the patient is excluded or declines	As for the other arms. Where the exclusion is penicillin allergy or an unpalatable suspension, move to the macrolide arm rather than referring, provided its criteria are met.

The medicine

Name, form and strength	Flucloxacillin 250mg/5ml oral suspension.
Legal category	POM.
Dose and frequency	● 3 months to 1 year (up to the second birthday): 62.5mg to 125mg four times a day for 5 days. (Under 3 months is excluded.) ● 2 to 9 years: 125mg to 250mg four times a day for 5 days. ● 10 to 17 years: 250mg to 500mg four times a day for 5 days. ● **Four times a day means four times a day. A three-times-daily regimen is a common error and underdoses the child.**
Quantity supplied	One bottle sufficient for a 5 day course at the dose selected. No repeat supply under this PGD.

Maximum treatment period	5 days, extended to 7 only on clinical judgement, with the reason recorded.
Route and method	Oral, on an empty stomach.
Storage	Store the dry powder at room temperature. After reconstitution, refrigerate and discard after the period stated in the SPC.
Adverse effects	Common: nausea, diarrhoea, rash. Rare but serious: cholestatic jaundice and hepatitis, which may be delayed; anaphylaxis; severe cutaneous reactions; antibiotic-associated colitis.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given, and from whom. Where the patient is under 16, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment. • Patient name, address, date of birth, and the GP with whom they are registered. • **WHETHER THE IMPETIGO IS LOCALISED OR WIDESPREAD, AND BULLOUS OR NON-BULLOUS, with the number and size of lesions that led to that conclusion.** • **THE PENICILLIN ALLERGY HISTORY in the patient's own terms, and which arm was therefore used.** • **Where a macrolide was supplied to a child: THE WEIGHT IN KILOGRAMS, measured today, and the band it placed the child in.** • Pregnancy status where relevant, and how it was established. • Name, form, strength, dose, quantity and date of supply. • That hygiene advice was given, and that the patient was told topical and oral treatment are not combined. • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when treatment finished.
Disposal	Advise return of any unused suspension to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet and the hygiene advice in writing.
Counselling	• **Four times a day, on an empty stomach: an hour before food or two hours after.** • Finish the course even if the skin looks better. • **Come back for yellowing of the skin or eyes, dark urine or pale stools, even weeks after finishing.** • Impetigo is contagious. Wash hands after touching the lesions and after applying any cream. • Do not share towels, flannels or bedding until the lesions have crusted over or healed.

	● Keep the lesions covered where practical, and discourage picking or scratching. ● **Stay away from school or nursery until the lesions are crusted and dry, or for 48 hours after starting an antibiotic.** ● Keep fingernails short.
Follow-up	Come back if there is no improvement after 48 to 72 hours, if the lesions spread, or if the child becomes unwell.

Macrolide, for penicillin-allergic patients

Patient Group Direction for the supply of clarithromycin, or erythromycin in pregnancy, for widespread non-bullous impetigo, bullous impetigo, or failure of topical treatment, where the patient is penicillin-allergic or flucloxacillin is unsuitable.

This arm is the route for a penicillin-allergic patient who needs an oral antibiotic.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for the product supplied, the BNF or BNF for Children, and NICE NG153 on impetigo.
- ALL USERS MUST BE ABLE TO DISTINGUISH LOCALISED FROM WIDESPREAD IMPETIGO, and non-bullous from bullous. Which arm applies turns on it.
- All users must be able to take a reliable penicillin allergy history and to distinguish a true allergy from an intolerance.
- All users must be competent to weigh a child, because the paediatric macrolide dose is by weight and not by age.
- All users must know that topical and oral antibiotics are NOT to be combined for impetigo.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must understand capacity and consent, including Gillick competence and parental responsibility.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Treatment of widespread non-bullous impetigo, bullous impetigo, or impetigo that has failed topical treatment, where the patient is penicillin-allergic or flucloxacillin is otherwise unsuitable, in accordance with NICE NG153.
Inclusion criteria	• Aged 1 month and over. There is no upper age limit. • Widespread non-bullous impetigo, bullous impetigo, or failure of topical therapy after 48 hours. • **Penicillin-allergic (true allergy or documented intolerance), OR a child aged 3 months to 17 who will not take the flucloxacillin suspension. Record which applies. A penicillin-tolerant ADULT is NOT eligible for this arm: this PGD carries no adult flucloxacillin, and the absence of an arm is not 'flucloxacillin unsuitable'. Refer.** • **Where the patient is a child: WEIGHED TODAY, in kilograms, and the weight recorded. The dose is by weight, not by age.** • Able to take an oral medicine. • Consent obtained, from a person with parental responsibility where the patient is under 16 and not Gillick competent. • No exclusion criterion present.
Exclusion criteria: clarithromycin interactions and contraindications	**These are contraindications in the SPC, not cautions. Refer.** • **Taking SIMVASTATIN or LOVASTATIN. Clarithromycin raises their levels and the combination causes myopathy and**

	rhabdomyolysis. Do not supply, and do not advise the patient to simply stop their statin; that is a prescriber decision. ● **Taking COLCHICINE. Deaths from colchicine toxicity have been reported with the combination, particularly in older patients and those with renal impairment.** ● **Taking an ERGOT ALKALOID: ergotamine or dihydroergotamine. Acute ergot toxicity with vasospasm and tissue ischaemia has been reported.** ● **Taking TICAGRELOR.** ● **Taking ORAL MIDAZOLAM, LOMITAPIDE, IVABRADINE, RANOLAZINE, DOMPERIDONE or PIMOZIDE (clarithromycin SmPC contraindications).** ● **History of QT prolongation, congenital or acquired, or of ventricular arrhythmia including torsades de pointes.** ● Taking any other medicine known to prolong the QT interval, or with hypokalaemia or hypomagnesaemia. ● Known hypersensitivity to clarithromycin or to any macrolide. ● Severe hepatic failure with renal impairment.
Exclusion criteria: all other	● Under 1 month of age. Refer. ● **Pregnant, and erythromycin is not suitable or not available. Erythromycin is the macrolide of choice in pregnancy, there being more documented experience of its use. Do not substitute clarithromycin.** ● Breastfeeding, unless the choice has been discussed and recorded. ● **A child who cannot be weighed today. Do not estimate from age.** **Refer, do not supply, where any of the following applies.** ● **Systemically unwell: fever, malaise, lymphadenopathy, or the patient appearing unwell. Refer the same day.** ● **Signs of a more serious condition, in particular cellulitis: spreading redness, warmth, swelling, or pain beyond the lesions. Refer to hospital.** ● **Immunocompromised, and impetigo is widespread. NICE advises hospital referral.** ● **Bullous impetigo in a baby, meaning a child aged 1 year and under (NICE CKS). Refer or seek specialist advice.** ● **Recurrent impetigo, meaning frequent repeated episodes. Refer for swabbing and consideration of decolonisation, rather than treating again.** ● Around the eye, or involving the eyelid margin, where a topical product cannot be used safely and an ophthalmic opinion may be needed. ● **Diagnostic uncertainty, or a presentation that could be herpes simplex, eczema herpeticum, or a fungal infection.** ● **A course of antibiotic already supplied for this episode under this PGD. One course per episode; treatment failure needs reassessment and a swab, not a second guess.** ● Valid informed consent not obtained, or consent from a person with parental responsibility not available where required.
Cautions	**Take a proper penicillin allergy history. A patient labelled penicillin-allergic on the basis of childhood nausea is losing access to a better antibiotic, but this is not the consultation in which to unpick that. Record what the patient describes and use this arm.** Advise on gastrointestinal upset and taste disturbance, which are common with clarithromycin.

	Advise the patient to seek advice for severe or bloody diarrhoea, which may indicate Clostridioides difficile. Warn about rash, and to stop and seek advice for any blistering or peeling. **Where the patient takes any statin other than simvastatin or lovastatin, or warfarin, check the current BNF: dose adjustment or monitoring may be needed and referral may be the safer course.**
Actions if the patient is excluded or declines	Explain the reason plainly. Where the exclusion is an interacting medicine, say that the problem is the antibiotic and not the skin infection, and make sure the referral happens the same day so the impetigo is still treated. Give the hygiene advice regardless.

The medicine

Name, form and strength	Clarithromycin 250mg tablets, or clarithromycin 125mg/5ml or 250mg/5ml oral suspension for children. Erythromycin 250mg tablets or oral suspension, for use in pregnancy.
Legal category	POM.
Dose and frequency: CLARITHROMYCIN	**Adults, young people and children aged 12 to 17 years: 250mg twice a day for 5 days. May be increased to 500mg twice a day for severe infection, with the reason recorded.** **Children aged 1 month to 11 years, BY WEIGHT:** ● Under 8kg: 7.5mg/kg twice a day for 5 days. ● 8 to 11kg: 62.5mg twice a day for 5 days. ● 12 to 19kg: 125mg twice a day for 5 days. ● 20 to 29kg: 187.5mg twice a day for 5 days. ● 30 to 40kg: 250mg twice a day for 5 days. **Weigh the child. Do not estimate from age. The bands assume a child of average size and the weight is what selects the dose.**
Dose and frequency: ERYTHROMYCIN, in pregnancy	● Adults: 250mg to 500mg four times a day for 5 days. ● Aged 8 to 17 years: 250mg to 500mg four times a day for 5 days. ● **Erythromycin is the macrolide of choice in pregnancy because there is more documented experience of its use than for other macrolides.**
Quantity supplied	Sufficient for a 5 day course at the dose and formulation selected. No repeat supply under this PGD.
Maximum treatment period	5 days, extended to 7 only on clinical judgement where lesions are severe or numerous, with the reason recorded.
Route and method	Oral. Clarithromycin may be taken with or without food. Shake any suspension before use.
Storage	Store in accordance with the SPC for the product supplied. Refrigerate reconstituted suspensions where the SPC requires it and discard after the stated period.
Drug interactions	See the exclusion criteria: simvastatin, lovastatin, colchicine, ergot alkaloids, ticagrelor and QT-prolonging medicines are contraindications rather than cautions. Also check warfarin, other statins, digoxin, midazolam, ciclosporin, tacrolimus and antiepileptics against the current BNF and SPC.
Adverse effects	Common: abdominal pain, nausea, vomiting, diarrhoea, taste disturbance, headache, rash. Uncommon: candidiasis, raised liver enzymes. Rare but

	serious: QT prolongation and ventricular arrhythmia including torsades de pointes, hepatitis and cholestatic jaundice, Stevens-Johnson syndrome and toxic epidermal necrolysis, anaphylaxis, Clostridioides difficile colitis, hearing loss which is usually reversible.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given, and from whom. Where the patient is under 16, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment. • Patient name, address, date of birth, and the GP with whom they are registered. • **WHETHER THE IMPETIGO IS LOCALISED OR WIDESPREAD, AND BULLOUS OR NON-BULLOUS, with the number and size of lesions that led to that conclusion.** • **THE PENICILLIN ALLERGY HISTORY in the patient's own terms, and which arm was therefore used.** • **Where a macrolide was supplied to a child: THE WEIGHT IN KILOGRAMS, measured today, and the band it placed the child in.** • Pregnancy status where relevant, and how it was established. • Name, form, strength, dose, quantity and date of supply. • That hygiene advice was given, and that the patient was told topical and oral treatment are not combined. • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when treatment finished.
Disposal	Advise return of any unused medicine to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet for the product given, and the hygiene advice in writing.
Counselling	• **Clarithromycin: twice a day for 5 days. Erythromycin: four times a day for 5 days. Finish the course.** • A metallic or bitter taste is common with clarithromycin and settles when you stop. • **Come back for severe or bloody diarrhoea, a rash that blisters or peels, or yellowing of the skin or eyes.** • Impetigo is contagious. Wash hands after touching the lesions and after applying any cream. • Do not share towels, flannels or bedding until the lesions have crusted over or healed. • Keep the lesions covered where practical, and discourage picking or scratching. • **Stay away from school or nursery until the lesions are crusted and dry, or for 48 hours after starting an antibiotic.**

	• Keep fingernails short.
Follow-up	Come back if there is no improvement after 48 to 72 hours, if the lesions spread, or if you or your child becomes unwell.

Appendix 1: Which arm, in order

1. Is the patient systemically unwell, or is there cellulitis?	• **YES: refer, same day or to hospital. Stop here.**
2. Is it localised non-bullous impetigo?	• YES: consider hydrogen peroxide 1% as a P sale first. If unsuitable or ineffective, fusidic acid 2% cream, three times a day for 5 days. • NO: go to 3.
3. Widespread, bullous, or topical treatment has failed. Is the patient penicillin-allergic?	• NO, and aged 3 months to 17 years (under 18): flucloxacillin oral suspension, four times a day for 5 days. • **NO, and an adult: refer. This PGD has no adult oral flucloxacillin arm.** • YES: go to 4.
4. Penicillin-allergic. Is the patient pregnant?	• NO: clarithromycin. Adults and 12 to 17 years, 250mg twice a day for 5 days. Children 1 month to 11 years, by weight. • YES: erythromycin 250mg to 500mg four times a day for 5 days. • **Check the clarithromycin contraindications first: simvastatin, lovastatin, colchicine, ergot alkaloids, ticagrelor, QT prolongation.**
5. In every case	• **Do NOT combine a topical and an oral antibiotic.** • Give the hygiene advice. • One course per episode. Treatment failure needs a swab, not a second guess.

Appendix 2: Key references

- NICE NG153: Impetigo, antimicrobial prescribing, published 26 February 2020.
- NICE Clinical Knowledge Summary: Impetigo, including hygiene measures.
- Summary of Product Characteristics for fusidic acid 2% cream, flucloxacillin 250mg/5ml oral suspension, clarithromycin and erythromycin, current versions.
- British National Formulary and BNF for Children.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, https://www.nice.org.uk/guidance/mpg2

Authorisation

Version	v011
Supersedes	Version 010, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this	

document.	

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

GET REAL HEALTH

Change history to version 004

Later versions are recorded in the Version and change record at the end of this document.

Version	v004
Date	8 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. ● The requirement for intact or only minimally broken skin is restored as an inclusion criterion of the TOPICAL FUSIDIC ACID arm, where it belongs. It was in version 001 and was lost in the version 002 rewrite. It is deliberately NOT applied to the oral arms: broken skin is a reason to use an oral antibiotic rather than a topical one, so applying it there would exclude the patients who most need them. Found by comparing against the archived version 001. ● A macrolide arm is added for penicillin-allergic patients: clarithromycin, or erythromycin in pregnancy, per NICE NG153. Version 001 authorised fusidic acid and flucloxacillin only, and its flucloxacillin arm excluded penicillin allergy, so a penicillin-allergic patient with widespread or bullous impetigo had no route at all. ● Full paediatric clarithromycin weight bands stated, from under 8kg to 30 to 40kg, with a requirement to weigh the child today rather than estimate from age. ● Erythromycin named as the macrolide of choice in pregnancy, with clarithromycin excluded there. ● Clarithromycin contraindications handled as exclusions rather than cautions: simvastatin, lovastatin, colchicine, ergot alkaloids, ticagrelor, and any history of QT prolongation or ventricular arrhythmia. ● Course length corrected to 5 days throughout, extended to 7 only on clinical judgement with the reason recorded. Version 001 gave 5 to 7 days as the default. ● An explicit instruction not to combine topical and oral antibiotics, which NICE states and which was absent. ● Hydrogen peroxide 1% named as the NICE initial option for localised non-bullous impetigo, and identified as a P medicine that does not require a PGD, so it is offered as a pharmacy sale rather than added as an arm. ● Mupirocin identified as the NICE alternative topical where fusidic acid resistance is suspected, and expressly NOT authorised by this PGD, so that a pharmacist reaching for it refers instead. ● Shared exclusions added across all three arms: systemic illness, cellulitis, immunocompromise with widespread disease, bullous impetigo in a baby, recurrent impetigo, eye involvement, diagnostic uncertainty including herpes simplex and eczema herpeticum, and one course per episode. ● Recorded as an unresolved limitation rather than fixed: the flucloxacillin arm remains a paediatric oral suspension for ages 3 months to 18 years, so there is still no adult oral flucloxacillin. A penicillin-tolerant adult needing an oral antibiotic is referred.

	• Records must now carry whether the impetigo was localised or widespread and bullous or non-bullous with the findings behind that, the penicillin allergy history, and the measured weight where a child was given a macrolide.

Previous versions

001, 1 November 2025	Patient Group Direction for the supply of flucloxacillin 250mg/5ml oral suspension for widespread non-bullous impetigo, bullous impetigo, or failure of topical treatment, in children aged 3 months to 17 years inclusive (under 18).

Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
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Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

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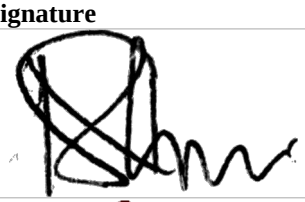
Version and change record
Version: v011
Issued: 30 September 2026
Supersedes: Version 010, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v010
Issued: 14 September 2026
Supersedes: Version 009, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v009
Issued: 11 September 2026
Supersedes: Version 008, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Flucloxacillin dose table: the first band now states that '3 months to 1 year' runs up to the second birthday, so a child aged 1 has a band.
'Bullous impetigo in a baby' defined as a child aged 1 year and under, per NICE CKS, in all three arms.
Duplicate v004 change history bullet removed.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v008
Issued: 11 September 2026
Supersedes: Version 007, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
The 'What changed, and why' block is removed from the cover and the document carries one validity statement and one dated signature page.
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v005
Issued: 10 September 2026
Supersedes: Version 004, 9 September 2026

Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.
Version: v006
Issued: 10 September 2026
Supersedes: Version 005, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.
Clarithromycin arm: oral midazolam, lomitapide, ivabradine, ranolazine, domperidone and pimozide added by name as SmPC 4.3 contraindications (none is QT-prolonging in a way the catch-all caught), and hypomagnesaemia added alongside hypokalaemia.
Version: v007
Issued: 10 September 2026
Supersedes: Version 006, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Flucloxacillin arm age band: '3 months to 18 years' both included an 18 year old and, two lines later, excluded 18 and over, with no dose band for 18. Now 3 months to 17 years inclusive (under 18). The dose table started at 1 month while inclusion started at 3 months; it now starts at 3 months.
Macrolide arm: 'flucloxacillin unsuitable' is defined narrowly (true penicillin allergy, documented intolerance, or a child who will not take the suspension) and the arm now states that a penicillin-tolerant adult is not eligible and is referred, matching the cover and Appendix 1.

Signed on behalf of Get Real Health

Version v011, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.